

Policy on Day-to-Day Leadership of the Organisation

«Hospital Name»

Document control

Document No.	HCO/ROM/POL/04	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Medical Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» is led by a qualified and experienced person who holds the organisation's day-to-day accountability. That person's identity, qualifications and legal compliance responsibilities are documented. Every department and programme has an identified leader, and the organisation's leader is formally reviewed for effectiveness every year.

1. Purpose

This policy explains how «Hospital Name» appoints a person with the right qualifications and experience to lead the organisation on a day-to-day basis; keeps the hospital compliant with all applicable laws, regulations and notifications; appoints leaders for each department and service; ensures every programme and department has effective leadership; and reviews the performance of the organisation's leader for effectiveness.

This policy does not cover patient assessment, clinical care, medication management, patient rights, infection control, or quality and safety monitoring — those are covered in other hospital policies.

2. Scope

This policy applies to those responsible for governance, the Medical Superintendent as the person heading the organisation, departmental leaders, the Quality Coordinator, and all staff at «Hospital Name» who hold named leadership roles or are involved in the appointment of department leaders.

3. Policy standards

«Hospital Name» appoints a qualified and experienced leader to head the organisation, keeps the hospital legally compliant, ensures every department has an identified leader, maintains effective leadership across all programmes and departments, and reviews the organisation's leader annually.

Staff follow the written guidance below, keep the records it requires, and raise concerns through the channels it describes.

4. Non-negotiable rules

1. Do not appoint a person to head the organisation without first verifying and recording their administrative qualification and experience — this check must happen before the appointment is confirmed, not after.
2. The hospital must maintain a current legislation register — the leader cannot plead ignorance of applicable requirements because the register was not kept up to date.
3. The legislation register must be actively maintained — the hospital must have a working mechanism for regularly picking up and incorporating amendments; a register that was created once and never reviewed since is not sufficient.
4. Every programme, service, site or department must have an identified leader on file — a gap must be escalated the same week it is found, not left open.
5. Each department leader must document the services their department provides — a verbal summary is not sufficient.
6. The organisation's leader must be formally reviewed for effectiveness at least once a year by those responsible for governance — a year without a review is a gap.
7. Staff who see any of these rules broken report it the same shift to the department in-charge or the Medical Superintendent.

5. What we do

5.1 Appoint a qualified and experienced leader

The person heading «Hospital Name» holds a qualification in hospital management or administration and has administrative experience in a hospital. Those responsible for governance verify both at the time of appointment and record that check. When the role-holder changes, the verification is repeated before the appointment is confirmed.

The specific level of qualification and the minimum years of experience are for those responsible for governance to define. The Guidebook specifies that 'appropriate qualification' means qualification in hospital management or administration, and 'appropriate experience' means administrative experience in a hospital.

Qualification certificates and a current CV are held on file.

5.2 Comply with applicable laws, regulations and notifications

The leader of «Hospital Name» is responsible for compliance with all applicable legislations, regulations and notifications, and takes active measures to adhere to them. The hospital operates as a duly permitted legal entity under all relevant registering authorities.

The leader maintains a current applicable-legislation register specific to this hospital's legal form and the services it actually runs. The register is not a copy of every central Act — it is a working list of the requirements that apply to this hospital. A mechanism exists to regularly update the register when amendments are issued, and statutory documents are updated within the timelines the relevant laws and registration authorities require. Where the hospital conducts research, including clinical trials, that research is conducted in accordance with statutory norms.

The form the update mechanism takes is for the hospital to determine — one possible approach is a tracker that logs each known requirement and flags pending updates.

5.3 Appoint department leaders

The leader appoints, or actively participates in recruiting, the leaders for each department and service that assist the day-to-day functioning of «Hospital Name». This applies at a minimum to the next level of the organisational structure. Appointment files show the leader's involvement in each decision.

Each department leader documents the services their department provides and has an integrated approach to aligning those services with the rest of the hospital. Department leaders participate in the hospital's patient safety and quality improvement activities in line with institutional priorities.

A department found without an identified leader is an open gap — this is escalated to the Medical Superintendent the same week.

5.4 Ensure effective leadership across all programmes and departments

The leader ensures that every programme, service, site or department at «Hospital Name» has effective leadership: a named person, an allocation of time for the leadership duties, and a clear reporting line. The organisation maintains a current leadership map recording these details.

Department leaders must have relevant qualification and/or experience for the area they lead, and domain knowledge of that department. The organisation develops and uses metrics to assess the effectiveness of its leaders — what those metrics are is for the organisation to determine.

When a leadership position becomes vacant, interim coverage is arranged and recorded until a substantive appointment is confirmed.

5.5 Review the organisation's leader for effectiveness

The performance of the organisation's leader is reviewed for effectiveness at least annually. The review is carried out by those responsible for governance or the appointing authority.

The form the review takes is for the organisation to determine — defined key result areas or a formal performance appraisal are both acceptable approaches. Where those responsible for governance and the person heading the organisation are the same individual, self-evaluation is an acceptable form of review.

The review is dated, documented and filed. A year without a review is a gap.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and properly resourced.
Medical Superintendent	Owns the legislation register, leadership map, and day-to-day records for this policy.
Those responsible for governance	Verify the leader's qualifications at appointment; carry out or assign the annual performance review.
Quality Coordinator	Audits this policy periodically; keeps training records; supports governance packs and committee reviews this policy requires.
Departmental leaders	Maintain documentation of their department's services; participate in quality improvement activities.

7. Quality monitoring

The Quality Coordinator audits this policy periodically, checking a sample of records against each of the What-we-do steps.

CORE objective elements show no critical gaps in the sample.

If an audit finds a gap that is still open after a defined timeframe, the hospital carries out a root-cause analysis and records the findings and corrective actions.

This policy is reviewed every year, and sooner if the legislation register, the leadership structure, or the governance review process changes significantly.

8. Training and staff acknowledgement

Staff covered by this policy are trained when they join, and again every year after that. Training covers the What-we-do steps and the non-negotiable rules.

I have read the Policy on Day-to-Day Leadership of the Organisation of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

9. Distribution

Medical Superintendent; those responsible for governance; Quality Coordinator; departmental leaders; staff covered by this policy

10. Abbreviations

Abbreviation	Meaning
CORE	Core objective element (NABH)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element
ROM	Responsibilities of Management (NABH Hospitals 6th Edition chapter)

11. Traceability to NABH HCO Full Accreditation 6th Edition ROM.4

This table is an index. It is not how the policy is organised.

OE	Level	Requirement	Where addressed	Responsible
ROM.4.a	Commitment	The person heading the organisation has requisite and appropriate administrative qualifications and experience.	Section 3; 5.1	Medical Superintendent
ROM.4.b	CORE	The leader is responsible for and complies with the laid-	Section 3; 5.2	Medical Superintendent

		down and applicable legislations, regulations and notifications.		
ROM.4.c	Commitment	The leader appoints / participates in the recruitment of department leaders of the organisation who will assist in the day-to-day functioning of the organisation.	Section 3; 5.3	Medical Superintendent
ROM.4.d	Achievement	The leader ensures that each organisational programme, service, site or department has effective leadership.	Section 3; 5.4	Medical Superintendent
ROM.4.e	Achievement	The performance of the organisation's leader is reviewed for effectiveness.	Section 3; 5.5	Medical Superintendent

12. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

ROM.4.a — Leader qualifications

- Qualification certificates and CV of the person heading the organisation, on file.
- Record of the governing body's qualification and experience check at appointment (or when the role-holder changes).

ROM.4.b — Statutory compliance

- Current applicable-legislation register specific to this hospital's legal form and services.
- Evidence of compliance against each item on the register.
- Dated review or update record showing amendments were captured and statutory documents updated within required timelines.

ROM.4.c — Department leader appointment

- Appointment file for each department leader showing the organisation-head's involvement in the decision.
- Current list of department leaders with appointment dates.
- Each department leader's documented service scope for their department.
- Escalation record for any department found without an identified leader.

ROM.4.d — Effective departmental leadership

- Leadership map naming a person, time allocation and reporting line for each programme, service, site or department.
- Dated review of the leadership map.
- Vacancy-coverage record for any leadership gap.
- Effectiveness metrics the organisation uses for its leaders.

ROM.4.e — Leader performance review

- Dated annual performance-review record for the organisation's leader.
- Review criteria or key result areas used.
- Filed outcome and any follow-up action.

13. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Accreditation Standards for Hospitals, 6th Edition (January 2025) — Responsibilities of Management, standard ROM.4.
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — ROM.4 interpretations.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and

technical content reflects recognised national and international guidance current at the date of preparation.

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